

Name : Mrs. Vishalakshi NBhat

PET No : PT254455

Age : 53yrs

Sex : Female

Ref/Dept : Dr.Vishal Kulkarni [DM(Med Onco)]

Date : 17/07/2025

PET-CT Report

Clinical indication: Known case of metastatic left breast carcinoma post- lumpectomy in 2019, and post radiation completed in 2020 . Post 3 cycles of chemotherapy completed on 28/06/25. Latest PET-CT on 19/03/25 (done outside, not available for comparison).

Technique: Whole body images (vertex to mid thigh) were acquired in 3-D mode 60 min after i.v. injection of 3mCi of 18F-FDG using a dedicated DIGITAL Time Of Flight PET-CT scanner.FBS before tracer injection was 138 mg/dl.Water was used as negative oral contrast. IV contrast given. Reconstruction of the acquired data(OSEM + Hyper DLR) was performed so as to obtain fused PET-CT images in transaxial, coronal and sagittal views

Findings: Physiological tracer distribution is seen in the brain, myocardium, gut, kidney and urinary bladder.

Brain :The supra and infratentorial brain parenchyma appears normal and shows normal physiological FDG uptake with no focal abnormal FDG uptake

It may kindly be noted that all brain metastases may not be apparent on a PET CT scan and an MRI head may be performed wherever clinically indicated.

Head and Neck:No discrete enhancing lesion or focal abnormal FDG uptake is seen in bilateral paranasal sinuses, oropharynx/ nasopharynx and hypopharynx including larynx and the parapharyngeal and paralaryngeal soft tissue.

No significant FDG avid cervical lymph nodes seen.

Both the lobes of the thyroid gland appear normal in size with no hypodense lesions or abnormal FDG uptake.

Breasts: Status post left breast lumpectomy. No discrete enhancing mass lesion or focal abnormal FDG uptake noted in the residual left breast parenchyma.

Right breast parenchyma appears normal in attenuation with no mass lesion or focal abnormal FDG uptake.

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No significant FDG avid axillary group of lymph nodes noted on either side.

Thorax: The heart and mediastinal vascular structures appear normal. The trachea and both main bronchi appear normal.

No significant abnormality is detected in the left lung parenchyma. No evidence of soft tissue density nodules/ focal abnormal FDG uptake in left lung parenchyma.

No significant interval change in the metabolic activity of multiple enhancing pleural nodules noted involving the right parietal pleura, mediastinal pleura and diaphragmatic pleura SUVmax along the superior mediastinal pleura is 7.5 compared to previous 6.6.

Mild right pleural effusion with subsegmental atelectasis of right lung fields noted.

Interval mild reduction in the metabolic activity with no significant change in the size noted involving the right hilar, subcarinal, prevascular and right lower paratracheal mediastinal group of lymph nodes, largest in the right hilar region approximately measures presently 1.4 x 1.2 cm, SUVmax 4.9 compared to previous 1.2 x 1.1 cm, SUVmax 8.8.

Abdomen and Pelvis: Esophagus and stomach appears unremarkable with no mural thickening / mass lesion.

Small and large intestine appear normal with no mural thickening/mass lesion/focal abnormal FDG uptake.

Liver is enlarged in size measures around 19.7 cm and shows diffuse fatty infiltration with no evidence of focal hypodense lesion / FDG uptake. No evidence of IHBRD.

Gall bladder is unremarkable. No evidence of gall bladder calculi.

No enhancing mass lesion/focal abnormal FDG uptake seen in the pancreas. No dilatation seen in the main pancreatic duct.

Spleen appears normal in size with no evidence of focal FDG uptake/hypodense lesions.

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Bilateral adrenal gland appears normal in size, morphology and show no nodularity/focal abnormal FDG uptake.

Bilateral kidneys appear normal in size, morphology and show no nodularity/focal abnormal FDG uptake

Aorta and IVC show normal CT anatomy

Tiny calcified posterior myometrial fibroid noted. Uterus and cervix appear normal in size with no enhancing lesion/focal abnormal FDG uptake. Bilateral adnexa appear unremarkable.

Urinary bladder is normal in shape size, and distension.

External genitalia appear unremarkable

No significant FDG avid bilateral inguinal lymph nodes seen.

No evidence of free fluid in the abdomen and pelvis.

No significant FDG avid mesenteric / retroperitoneal / pelvic lymph nodes seen.

Musculoskeletal system: Visualized bones of the skeleton show mild degenerative changes and otherwise appear normal with no discrete lytic / sclerotic lesion / focal abnormal FDG uptake.

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Impression: Known case of metastatic left breast carcinoma post- lumpectomy in 2019, and post radiation completed in 2020 . Post 3 cycles of chemotherapy completed on 28/06/25. Latest PET-CT on 19/03/25 (done outside, not available for comparison).

- No definite evidence of local recurrence noted in the residual left breast parenchyma
- No significant interval change in the metabolic activity of enhancing pleural deposits along the right lung pleura with right mild pleural effusion and sub segmental atelectases of underlying lung as described above
- Mild interval reduction in the metabolic activity with no significant change in the size of mediastinal group of lymph nodes as described above
- No other focal abnormal hypermetabolic lesion noted elsewhere in the present study
- Hepatomegaly with fatty infiltration
- Other findings as described above

Previous PET-CT CD not available for the comparison, present scan findings response evaluation is based on the previous PET CT report dated 19/03/25.



Dr. Rashmi Angadi

Director–Nuclear Medicine, PET-CT &Theranostics

Thanks for the referral

Note: Investigations have their limitations. Solitary investigation never confirms the final diagnosis of disease. Kindly bring this report and CD of the PET CT scan for comparison in the follow up visit

VISHALAKSHI N BHAT 53Y/F

PID:97254455

F 53Y

NeuMod Digital PET-CT

17/07/2025

UNITED
IMAGING

VISHALAKSHI N BHAT 53Y/F PT254455

18F-FDG Body PET

Image Fusion

20250717

NeuMod Digital PET-CT

UH uM 550

